

Key Takeaway: Strengthening your system and your employment

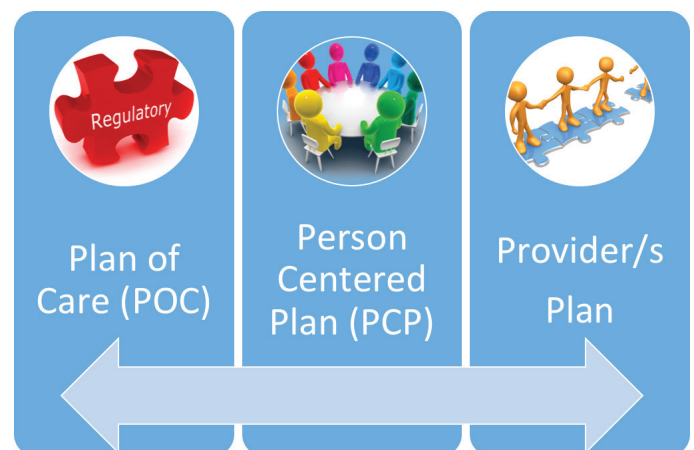
Guest presenter: Robin Cooper, director of technical assistance, NASDDDS

In recent years, new regulations and expectations for individual service plans have emerged which have changed the relationship between state and local systems and provider organizations. Additionally, with the advent of conflict-free case management, states are making changes to various roles and responsibilities, including who ensures that individual plan requirements are met and keeps the individual's support team aligned with what is important to individuals and families. Individual service plans require personalized, customized approaches to supporting people to access a community-based life.

Why is this relevant to SELN member states? These systems changes impact how well a state succeeds in achieving new employment outcomes. Member states can reach out to each other to learn more about various activities, such as:

- » How are states bringing employment into the conversation during the planning process and before provider or service plans are developed? In one state, this starts with a guided conversation on employment at intake, with a strong person-centered focus, even before eligibility or any services are discussed.
- » If states are transitioning to an electronic plan format, how does that impact how many plan components are developed? What are states experimenting with to address challenges? Does the new format change the relationship with other state systems, and how are the results looking?
- » Are states vigilant in helping individuals and families understand all the interconnected pieces of planning, using plain language and family-friendly examples of planning documents and processes?

- Because there are often multiple state system layers (a single state Medicaid agency as well as a state intellectual and developmental disability agency) in addition to regional, area, and local players, confusion emerges related to who is doing what. The notion of a “single service plan” may have arisen out of a desire to have all the important pieces in one place, and to assure funders, authorizers, and monitors that all aspects of individual service planning have been covered.
- State policies and procedures must address who is responsible for ensuring all aspects of individual planning are addressed, maintaining oversight, deciding how often plans are revisited, and being sensitive to individuals' and families' understanding of the process. The plan of care, person-centered plan, and provider plan are created and maintained in different places, but together describe all the aspects of the full individual service plan. In the end, each state must have a clearly described process ensuring that all aspects of individual planning are addressed and completed.



- The plan of care (POC) is a picture of authorized and reimbursable supports and services, representing what the person receives, and ensuring that billing and authorizations match. Federal regulations for Medicaid-funded services

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[Meeting Recording](#)

dictate what must be included on the POC. For state-funded services, look to the particular state's policies and procedures. The POC in this context includes signatures, has required fields, and is typically used in an authorization process. As services change, the POC is updated. It may reference expected services (e.g., employment), including a future start date; this reference would be removed in updates of the POC if it does not transpire.

- The person-centered plan (PCP) reflects the details: the person's gifts and concerns, and what is important to and important for information. A variety of team members are involved, and the PCP should evolve over time and through multiple meetings and discussions. Federal regulations do not stipulate how the PCP process must work, and states have latitude to determine this. Many states have made significant investments in person-centered planning and

thinking to ensure that this process reflects how the paid support system will help the individual access their desired services and supports.

- A provider's detailed plan outlines specific, measurable tasks and activities that will occur to get to the outcomes noted in the PCP and in accordance with the services authorized in the POC. The provider organization outlines what services will help the person achieve the stated outcomes. More than one provider may interface with an individual, leading to more than one detailed provider plan. A recommended practice is to share (with permission) pertinent information from the PCP process with providers, to inform development of the provider plan and assist in structuring supports. Service coordinators and case managers are often charged with keeping all the pieces in sync.

Resources

[FAQ on competitive integrated employment](#)

[Ohio's Imagine system](#)

[Washington Post: After 40 years, U.S. court ends supervision of D.C.'s care for mentally disabled citizens](#)



State Employment Leadership Network

The SELN is a place for states to connect, collaborate, and create cross-community support regarding pressing employment-related issues at state and federal levels for individuals with developmental disabilities.

The SELN was launched in 2006 as a joint program of the National Association of State Directors of Developmental Disabilities Services and the Institute for Community Inclusion at the University of Massachusetts Boston.



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